



CEI RESEARCH: Ambulatory Strategy in Mexico

Business Development Department
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A CEI strategy to move beyond CHRISTUS Muguerza's current CAM footprint and build a purpose-built, hospital-adjacent, digitally enabled, payor-connected ambulatory platform.



Contents

1. Sources of Information
2. Executive Summary
3. Competitive Benchmark: What's Done The Right Way
4. Opportunity: Market Logic, Current State & Strategic Rationale
5. The CEI Ambulatory Model: Strategy & Proposal
6. Inorganic Growth: Saltillo Clinic, Roadmap and Business Logic
7. Technical Analysis and Requirements
8. Preliminary Economic Proposal: CAPEX & OPEX and ROI Analytics
9. Conclusions, Strategic Implications and Recommendations
10. Appendix



1. SOURCES OF INFORMATION

All the information related to Monterrey's macroeconomic profile, demographics, history, as well as the assumptions and comments presented in this document, is derived from various sources, including interviews, publications, reports, and journal articles gathered in our previous business cases analysis from Alivia Clínicas Especializadas, Oncare and Salud Digna. All sources are listed in this section:

1. Alivia Business Case Study – [Alivia Disruptive Model](#)
2. Oncare Biasness Case Study – [Oncare Healthcare integrated Model](#)
3. Salud Digna Business Case Study – [The McDonalds of Health](#)
4. Healthcare Experiences Grow Patient Loyalty – [Accenture](#)

Comentado [JD1]: [@Saul Horacio Juarez Peña](#) este qué es? Lo que me platicaste de tu amiga?



2. EXECUTIVE SUMMARY

This document provides the culmination of 3 business case studies developed by the Business Development department at CHRISTUS CEI. This document encapsulates and provides an in-depth analysis of three major healthcare business models for outpatient services: Alivia Clinicas Especializadas, Oncare, and Salud Digna. All three pioneers of disruptive value propositions that have changed the Mexican ambulatory healthcare market, their models have pinpointed clear areas of opportunities for Muguerza's system which could represent an increase in Lives Served and hence, an expansion in our mission.

Each model represents a distinct approach to a shared structural problem: the gap between the current needs of patients covered by Private Healthcare Insurance (PHI) and what traditional hospital systems provide. ALIVIA has built a premium, insurance-integrated infusions platform with a strong physician referral engine and white-glove patient experience. ONCARE has scaled a tech-enabled ambulatory model with a physician-facing app that drives referrals beyond financial incentives, enhancing patients' treatment for ambulatory surgeries, lab and imaging centers and oncology services, all within one single ecosystem. SALUD DIGNA has demonstrated that Lean Manufacturing principles applied to healthcare can generate high-volume, low-cost diagnostics at scale, without sacrificing patient throughput or quality perception.

When we combine these three models, we shed light on a major opportunity for CHRISTUS Muguerza. The opportunity is not to build ambulatory care from scratch, nor to suggest that Muguerza lacks ambulatory assets. The system already has the hardest pieces in place: existing clinics and CAMs, hospital infrastructure, strong brand equity, physician relationships, and long-standing insurer connectivity. The real opportunity is to transform these existing assets into a competitive, standardized, digitally enabled ambulatory platform — one capable of capturing the outpatient volume that is currently leaking to specialized competitors such as Alivia, Oncare and other competitors.

Executive Snapshot: What has the competition accomplished?

Category	Alivia Clínicas	Oncare	Salud Digna
Model	Specialty ambulatory infusion & pharmacy platform.	Integrated specialty care network + ambulatory surgery.	High-volume diagnostics, lab, imaging & optics.
Active Clinics	24 clinics	6 specialized centers + 1 ambulatory hospital	235 clinics
Geographic Footprint	15 states	Monterrey & Mexico City.	32 states, 78 cities
Patient Volume	25,000+ patients / 200,000+ infusions (since inception).	N/A	18M patients / 28M services (2024).
Est. Annual Revenue	~USD \$55M (2023).	Not disclosed	~USD \$420M (2024E).
Target Segment	AB / C+ with private insurance.	AB / C+ with private insurance.	C- to C / B price-sensitive.
Key Alliances & Partnerships	35+ insurers (direct billing), 24 pharma partners, Farmacias Benavides ecosystem.	15+ insurers (direct billing), network of 100+ specialist physicians.	IMSS/ISSSTE diagnostic agreements, real estate and equipment lease model, optics supply chain partnerships.

3. COMPETITIVE BENCHMARK: WHAT'S DONE THE RIGHT WAY

3.1 Alivia Clinics: Ambulatory Infusion Therapies

Alivia is a multi-site outpatient specialty care platform that was built around an existing structural gap between the public healthcare system and the private healthcare dimension, in which patients with complex recurring conditions: oncology, rheumatology, MS, Crohn's, nephrology – that require high-cost infusion therapies but do not need full hospitalization. Rather than competing as a general clinic, Alivia positioned itself at the intersection of the infusion treatment patient's journey.

Key insights: What do they do right

1. **Payor integration as a competitive advantage:** With more than 35 insurer relationships and direct payment and billing integration, Alivia removes the biggest friction point in the private specialty care: administrative coordination with Private Healthcare Insurance (PHI) payors.
2. **Physician referral network via Soy Doctor and medication through Alivia RX:** Alivia has formalized its physician relationship model into a structured program that partners with 1,500+ specialists. Physicians refer patients, Alivia handles scheduling, medication coordination via their specialty pharmacy in partnership with Farmacias Benavides (Alivia RX), insurance pre-authorization even with direct payment, and administration. Physicians retain clinical authority. One referral generates a recurring patient relationship.
3. **Price advantage over hospitals:** Alivia self-stated that their ambulatory model is estimated to be up to 50% more affordable than hospital-based administration for the exact same treatment. Chemotherapy administration alone benchmarks at MXN \$3,150 at Alivia vs. MXN \$7,500 at Mugerza a 4x gap that is compelling for both patients and payors.
4. **WhatsApp – automated patient coordination:** Their entire patient intake is through their automated WhatsApp, including scheduling, location assignation and insurance paperwork, direct quotation is made through this channel, reducing friction and enabling fast response without complex digital infrastructure.
5. **Vertical integration across the full treatment journey:** Alivia is not just an infusion site, it combines outpatient administration, specialty drug access (Alivia RX), and insurance management under one roof. This allows the company to capture margin at multiple points of the treatment cycle, not just at the clinical visit.
6. **Modernism & comfort: a complete infrastructure:** Alivia clinics are specifically designed to not feel like a hospital. Reclinable chairs, private infusion rooms, curated aesthetics, natural light, and a calm environment are core to the patient's experience, delivering an experience through **Dignity** and **Compassion**.

Main takeaway for our ambulatory strategy:

The real opportunity in the ambulatory infusion therapies market is not the clinics themselves. These are the initial catch, but the retention factor is the coordination layer, **whoever controls the insurer relationship, the physician referral flow, clinical records consolidation and the medication pathway controls the patient**. Mugerza already has the brand and is a well-known name for the insurer companies. The missing piece is the **direct payment & alliances**, and the strengthening of the **physician referral benefits** (not financial but through digital tools such as Soy Doctor App and insurance paperwork to incentivize direct payments). Finally, the defining factor that we take away from Alivia is their UX, **the modernism and sense of comfort that their clinics offer is a key differentiator**.

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3.2 Oncare: The Northern Mexico Integrated Healthcare model

Oncare is a complete healthcare system that captures and exploits the entire patient journey even before it starts, from detection to treatment and follow-up until completion and the option to return at any moment without any friction. Oncare connects all of them under a single coordinated system that englobes several specialties and offers treatment to all of them from preventive check-ups and early detection to ambulatory infusions, hemodialysis, endoscopy, and minimally invasive surgery. With 6 specialized centers, 100+ affiliated physicians across 15+ specialties, and 45,000+ patients served.

Key insights: What do they do right

1. **Full patient journey integration:** Oncare is the only private outpatient platform in Nuevo León that combines early detection and ambulatory treatment across oncology, hematology, renal care and pulmonology, and a short stay surgical hospital. All operating as a single coordinated system. Every stage feeds the next, eliminating the administrative and logistical fragmentation tax that patients, physicians, and insurers absorb in traditional hospital-centric models.
2. **Open physician network:** Besides their salaried staff that manage their specialized centers, Oncare structured itself as a healthcare operating system for independent specialists. Over 100 physicians use Oncare's infrastructure insurance integration, and digital platforms to treat their patients. The value proposition for physicians is not primarily economical, it relies on the operational advantages that Oncare provides, high-end diagnostics and treatment technology without individual CAPEX expenses, administrative concierge for insurance paperwork, faster payment collections, and a digital platform to manage their patients in real time.
3. **Dual digital Infrastructure:** Oncare has a key differentiator – their customer relationship channel is not a single app, is a dual platform shaped by their medical app “[Oncare medical record](#)” a physician faced administrative channel where doctors can manage their patients, plus their “[Oncare Patient Portal](#)”, a centralized platform for patients with all their clinical information and documentation. These two platforms create a closed-loop ecosystem where clinical data, administrative workflows, and insurance tracking join into a single experience for both doctors and patients.
4. **Payor connected access:** Direct alliances with a variety of top insurance companies like GNP, MAPFRE and MetLife allow patients to access high-specialty services without initial out-of-pocket expenses or reimbursements. By managing preauthorization and insurance paperwork, Oncare reduces the friction that often happens between the patient, the insurer and the physician.
5. **Multi-channel branding:** Oncare has maximized their current infrastructure, they have evolved beyond its core oncology brand by hosting independent specialty groups – The Gastro center and Pulmonary Center – Under different brands that share Oncare facilities. These fill their inactive capacity in surgical suites and recovery rooms while allowing these other targeted patients to get the same quality service, without the psychological barrier of a dedicated cancer center.

Main takeaway for our ambulatory strategy:

Physician loyalty is not bought, it is achieved through infrastructure access, digital tools and administrative enablement. Oncare shows that their physician app and open staff models are effective mechanisms that convert independent specialists into a captive referral network. Muguerza already has clinical credibility and specialist relationships. **The missing piece is a structured digital layer**, a physician-facing platform that makes Muguerza's ambulatory infrastructure the easiest, most

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administratively seamless environment for a specialist, **we need to achieve clear incentives that allow us to gain loyalty** with our own tenants.

3.3 Salud Digna: Operational Excellence: “The McDonalds of healthcare”

Salud Digna is the largest private diagnostics network in Mexico; they developed a structurally robust healthcare business model that has been regarded as one of the best healthcare business models in Latin America and has been a topic of research by Harvard Business School. Founded in 2003 in Sinaloa by Jesus Viscarra, a former industrial executive that built Salud Digna under a single conviction: The constraints limiting medical diagnostics in Mexico were not medical – they were operational.

Key insights: What do they do right

1. **Lean manufacturing applied to healthcare:** Every Salud Digna clinic runs like a factory: Standardized protocols, parallel task execution, maximized patient flow, and real time performance monitoring. While a physician examines one patient the other one is being prepared. Every process is documented into checklists. Every clinic manager reviews four operational quadrants daily via a Personal Digital Assistant. This is not efficiency as a goal; it is efficiency as the operating model.
2. **Hub-and-spoke infrastructure:** MRI and CTE scanners sit in their Sinaloa and Mexico City Hubs, while lighter spoke clinics generate patient volume and feed them into high capex nodes, maximizing asset utilization and eliminating any double investing.
3. **Leased assets & equipment strategy – low CAPEX:** A key characteristic of Salud Digna’s model is that they don’t buy their equipment, they have a usage-based leasing agreements with Roche and GE Healthcare, amongst other providers. Also, Roche donates (Salud Digna is a Non-for profit organization) or sells their equipment at deep discounts in exchange for exclusive consumable supply chain contracts. Salud Digna is Roche's 6th largest client worldwide. GE HealthCare manages 140+ imaging devices across the network under a full lifecycle agreement. New clinics open faster, balance sheets stay lean, and expansion risk is shared with suppliers.
4. **Eyeglasses as their margin generator:** With approximately 50% gross margins, the optics business model is their financial engine that funds the heavy equipment like the MRI machines. The glasses are their jewel hiding in plain sight, they absorb lower margin diagnostics services and make the entire pricing model viable.
5. **WhatsApp as the primary UX:** 91% of all patient interactions, booking, payment, results delivery and appointment reminders, happen inside WhatsApp. No app download required. No call center dependency. In a country where low-income users have unlimited WhatsApp but limited mobile data, this is the front door to the entire operation.

Main takeaway for our ambulatory strategy

Salud Digna’s operating and economical model makes them the leading diagnostics service in Mexico. Their lean manufacturing philosophy and their hub-and-spoke network, and asset-light CAPEX structure are what make pricing, scale, and speed of expansion possible. For CHRISTUS, the direct application is not to replicate Salud Digna's diagnostic model – **It is to borrow its infrastructure logic: standardize ambulatory clinic operations**, deploy a hub-and-spoke footprint anchored on existing Muguerza facilities, **combined with a replication of their complete all around WhatsApp model.**

4. OPPORTUNITY: MARKET LOGIC, CURRENT STATE & STRATEGIC RATIONALE

The Competitive benchmark presented in the previous section shows a clear conclusion: the ambulatory specialty market in Mexico is being captured by those who solved problems that CHRISTUS Muguerza has historically left unaddressed, which can be attributed to lack of innovation – Nevertheless, Muguerza has all the necessary capabilities to make these changes. We just need the correct action plan and business model to shift from hospital-based inpatient care delivery to a complete combination of both ambulatory service and the existing ones.

CHRISTUS Latam is no stranger to ambulatory business models, UC CHRISTUS has its own fully specialized “UTM’s”; lab & imaging centers specialized in sample collection and ambulatory services, with more than sixty-six operational ambulatory units all around the territory, plus an optics department at San Carlos de Apoquindo Hospital. In Mexico, Muguerza has CAM’s: an ambulatory attempt by CHRISTUS, that needs work but serve as an initial step for our fully integrated ambulatory model.

CHRISTUS Muguerza has this clear opportunity to enter the ambulatory market, not by optimizing what it already has, but by implementing an already existing model, that has shown success throughout the Mexican territory. These are the two converging factors that make this proposal a real opportunity:

1. Demand is already there and is currently being captured by others, the specialty outpatient population in Nuevo León and the broader Northern Mexico region is not hypothetical. It exists today. A meaningful portion of it is already being served by standalone ambulatory operators, not because those operators are clinically superior to Muguerza, but because they offer a more convenient, seamless, more affordable, and less administratively burdensome experience for recurring treatment. Every recurring infusion patient treated at a competitors’ facility is a patient relationship – and a medication margin, an insurer touch point, and a referral stream – that does not return to Muguerza.
2. The insurer incentive is structural and growing. Major Private Healthcare Insurance (PHI) payors are under sustained financial pressure from specialty care claims inflation. Ambulatory administration of chemotherapy, biologics, and infusion therapies at a dedicated outpatient facility reduce the total cost of claims significantly versus hospital-based administration. Insurers have a direct financial incentive to steer patients toward certified ambulatory providers. Muguerza's existing payor relationships – already active, already trusted – are the fastest path to becoming the preferred ambulatory destination for insured specialty patients in the region.

4.1 Strategic Rationale & Design Principles

CHRISTUS Muguerza enters this space with a set of clear structural advantages that none of the competitors here analyzed were able to start with. We aren’t reinventing the wheel; we already have a wide starting ground that we can exploit.

What CHRISTUS Muguerza has that no ambulatory operator can replicate:

- **Brand equity and clinical credibility:** Muguerza is the most recognized private hospital brand in Northern Mexico, dominating the market in one of the most affluent corridors of the nation, is already trusted by patients and has built relationships with physician and insurers for decades.
- **Physical infrastructure – A fully operational hospital adjacent to ambulatory units:** Patients who require escalation beyond ambulatory care can be moved upstairs. No transfer, no new

intake, no lost continuity. This is a clinical safety net that Alivia, Oncare, and every standalone ambulatory operator simply cannot offer.

- **Existing and growing Private Healthcare Insurance (PHI) payor connectivity:** Muguerza's years of active insurer relationships represent a pipeline that took Alivia a decade to build with 35+ payors. The ambulatory channel does not need to earn insurer trust from zero – It needs to extend and formalize existing relationships into a new, lower-cost care setting. Plus, our current projects with Insurance Brokers such as Sekura (Mexico's largest broker), position ourselves in the best path to develop specific products and strategies that increase efficiencies with patient-payor operations.

4.2 The current Hospital Experience & CHRISTUS Muguerza Current Ambulatory Platform

CEI'S goal is to redefine the current ambulatory attempt that Muguerza has created via their ambulatory clinics. The situation relies on the fact that the current model is contradicting the ambulatory services core business model. For instance, Muguerza's "Ambulatory Clinic" at San Pedro operates as separated entity but feels almost identical to the hospital counterpart, it appears to be a mini hospital instead of an ambulatory clinic.

The feel and aesthetics of the facility are cold and do not create a sense of safety for our patients. This "ambulatory clinic" that Muguerza has created is a vague attempt at competing against models that Muguerza has no idea how they work.

Muguerza CAMs are first contact units that offer general consultation, basic imaging and some specialty physician services. They are 8 CAM centers that operate as individual units that provide lower complexity access points outside hospitals. However, they lack the Alivia and Oncare look & feel, location, modernization, comfort and user experience. They function as independent clinics that try to address lower income deciles without the care of creating a complete operational model and a robust business model.

Muguerza does not lack ambulatory infrastructure; it lacks the modernization, plus digital and operational layers required to make that infrastructure behave like an integrated ambulatory platform.

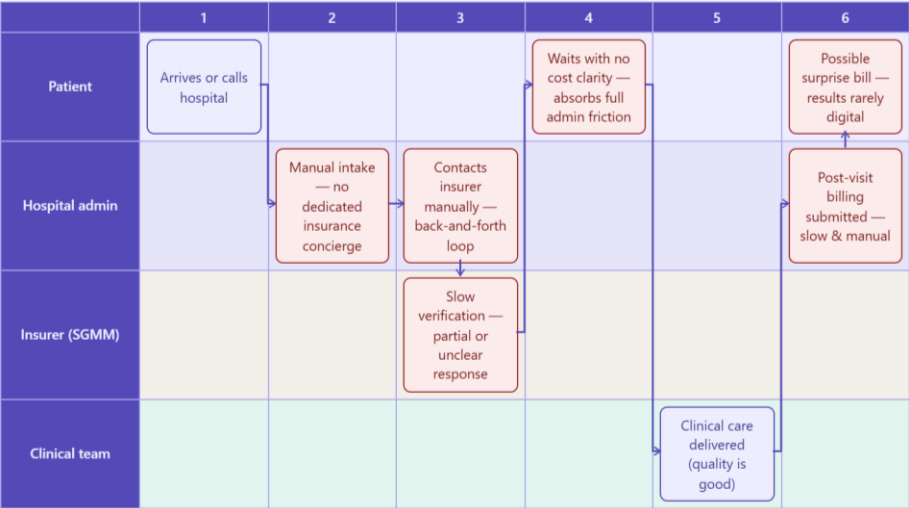
4.2.1 The Current Hospital Experience

A Muguerza patient today enters a well-equipped, top clinically recognized hospital. The care itself is not the problem. What breaks the experience is everything surrounding it. Muguerza's model is structurally designed for low volume and high price – a model that works for complex inpatient events but creates disproportionate friction for the recurring specialty outpatient who simply needs to show up, get treated, and leave.

The most consistent pain point is insurance coordination. When a patient presents Private Healthcare Insurance coverage, the process of validating the policy, confirming coverage scope, obtaining pre-authorization, and coordinating billing is slow, opaque, and largely absorbed by the patient. There is no dedicated concierge function – no specialized team whose sole job is to own that interaction with the insurer on the patient's behalf. The patient waits. The process is unclear. And in many cases, the out-of-pocket exposure at the point of care is uncertain until well after the clinical event.

Current journey

Current state — Muguerza patient journey (today)



Normal step Friction / pain point

5. THE CEI AMBULATORY PLATFORM: STRATEGY & PROPOSAL

Once we established the current ambulatory state for Muguerza, the Business development team started with the progress of the requirements needed to achieve a successful **Muguerza Ambulatory model**.

5.1 CEI Ambulatory Premises: What Are We Aiming For?

The ambulatory platform CEI proposes for Muguerza is a purpose-built, hospital-adjacent, digitally integrated, payor-connected specialty care network molded to match Alivia and Oncare by utilizing the same principles that make this two the current leaders in ambulatory service clinics in northern Mexico.

It is designed to serve the AB/C+ Private Healthcare Insurance (PHI)-insured patient who requires recurring specialty treatment – Infusions, diagnostics, lab, imaging and ambulatory surgeries – but does not need full hospitalization. The model operates as a clinically and experientially distinct environment: it looks, feels, and functions like a premium specialty clinic, not a hospital department. It is connected to Muguerza's full hospital infrastructure for escalation, anchored on existing facilities to minimize capital deployment, and enabled by Muguerza Connect to eliminate the administrative friction that has historically driven recurring patients toward competitors.

This is not a new service line added to the hospital. It is a separate operating model with its own patient experience, its own physician engagement logic, its own insurer coordination infrastructure, and its own economics and digital infrastructure.

Why does this proposal move beyond CAMs?

The CAMs were not designed to compete against Oncare and Alivia. They function as general first contact units: lower complexity, lower acuity and structurally disconnected from the insurer integration and physician referral logic that drives recurring specialty volume. Three factors make building a new model the right decision to retrofit the existing one:

1. **Location:** CAMs are not positioned in the high-income corridors of the city, where our AB/C+ Private Healthcare Insurance target patients live and seek care. Relocating or rebuilding them defeats the asset-light logic.
2. **Investment:** The investment needed to transform these CAMs from generic cold ambulatory clinics to Alivia and Oncare level clinics is far beyond the initial investment needed for new ambulatory clinics inside Muguerza hospitals infrastructure, **or having new ones through a lease model.**

This proposal is not an upgrade of what exists. It is a new operating model – one that uses Muguerza's existing assets as its foundation, not its ceiling.

5.2 Growth Model – Organic and Inorganic

The CEI ambulatory strategy is structured around two complementary and sequentially executable growth models, design to balance speed and deployment ensuring, capital efficiency, market coverage, and the success of our new business model.

Model 1 – Organic Growth: Activation of existing space at Muguerza facilities as dedicated ambulatory zones. This model prioritizes speed, capital efficiency, and immediate leverage of existing infrastructure, brand, and insurer relationships. The ambulatory unit operates as a clinically and experientially distinct

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environment within the Muguerza hospitals – designed and operated as a specialty clinic, not a hospital department – with direct escalation access to full hospital capabilities when clinically required.

Model 2 – Inorganic Expansion: Asset-light hub-and-spoke deployment into new geographies, anchored on existing Muguerza hospital infrastructure as the hub. New spoke locations are structured as fully leased operations – space, equipment, and technology – reducing CAPEX significantly and avoiding permanence in one location if it doesn't work as expected. This model enables geographic expansion with significantly lower financial risk and faster time-to-market than greenfield hospital development.

- **Saltillo Hub & Spoke opportunity:** The Business Development team has identified Saltillo as the first target geography for inorganic expansion. The opportunity centers on an available greenfield site currently under analysis, which would be structured under a full lease model – covering the physical space and all clinical equipment – eliminating upfront CAPEX and aligning operational costs directly with revenue generation. The business logic is divided in two: the spoke – Saltillo greenfield site that serves as the primary patient facing ambulatory unit for the Saltillo market. Designed and equipped to give our patients the best possible experience. The spoke delivers the full ambulatory service offering – Infusions, laboratory, imaging – within a modern, purpose-built specialty clinic environment. And secondly, the Hub that would be the Hospital Muguerza Saltillo, which serves as the escalation net for our clinical unit where complex treatments can be escalated and processed with the highest standards.

Five key design principles:

These principles define what the CEI ambulatory model must be – and equally, what it must never become.

1. Clinic feel, not hospital feel: Every physical and experiential element of the ambulatory unit must signal that this is not a hospital. Curated aesthetics, natural light, reclining infusion chairs, private treatment spaces, and a calm, unhurried environment are not amenities – they are core to the model. The moment a patient feels like they are inside a hospital, the value proposition begins to erode.

2. First operations pre-authorization is completed before the patient arrives. A dedicated administrative team – whose sole function is insurer coordination – owns the entire relationship with the Private Healthcare Insurance payor on the patient's behalf. The patient's role is to show up. Every administrative friction point that currently falls on the patient is absorbed by the ambulatory operation.

3. Physician as the primary referral engine Patient acquisition in the ambulatory model does not come from marketing. It comes from physicians. The model is designed to make referring to Muguerza's ambulatory platform the easiest, lowest friction, most administratively advantaged option for the independent specialist. The physician refers. Muguerza's ambulatory team handles scheduling, pre-authorization, treatment coordination, and outcome reporting. The physician retains clinical authority and gains operational simplicity.

4. Hub-and-spoke on existing Muguerza infrastructure Capital efficiency is structural, not optional. Every spoke clinic is deployed on existing Muguerza real estate or under a full-lease model – space, equipment, and technology – with Hospital Muguerza serving as the clinical hub

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and escalation net. No greenfield hospital builds. No duplicated fixed costs. Expansion speed is a direct function of asset-light execution.

5. Muguerza Connect as the operational backbone Every patient interaction – booking, pre-authorization status, results delivery, appointment reminders, follow-up – flows through a single digital channel. Muguerza Connect, already in active beta, is the platform that connects the patient, the physician, the administrative team, and the insurer into one coordinated operating system. Friction disappears because the platform is designed to absorb it.

5.3 Muguerza Connect: What is it?

The fifth design principle established in the previous section defines “Muguerza Connect” as the operational backbone of the CEI ambulatory platform. This section explains what that means in practice.

Muguerza Connect is the clinical-operational CRM of our ambulatory platform proposal. It is designed to be the operating layer that connects patients, physicians, administrative teams, and insurers into one coordinated ambulatory platform managed by Muguerza's ambulatory team through the clinics.

The role of **Muguerza Connect** is to absorb the administrative friction that could otherwise weaken the new CEI ambulatory model in the same way it currently weakens the patient experience across hospitals and private medical offices at Muguerza. In today's model, patients are often met with small obstacles that create unnecessary friction. In a market where recurring specialty patients have real alternatives – Alivia, Oncare – small frictions create large leakage. [Accenture](#) found that 78% of patients who switched healthcare providers cited “ease of navigation” factors as a reason for leaving. These factors are visible in Muguerza's current digital and administrative model: scheduling, insurance paperwork, pre-authorization follow-up, document sharing, results tracking, and communication between disconnected parties.

The root cause is the lack of a structured administrative and digital infrastructure around Muguerza's independent physicians. Private tenants currently lack meaningful hospital-provided digital services. The system remains decentralized, manual, and traditional, with coordination still depending heavily on phone calls, manual scheduling, and fragmented communication. As a result, many physicians rely on third-party SaaS platforms, paid out of their own budgets, to manage patient tracking, scheduling, revenue, digital medical records, front-desk coordination, patient communication, and other operational needs.

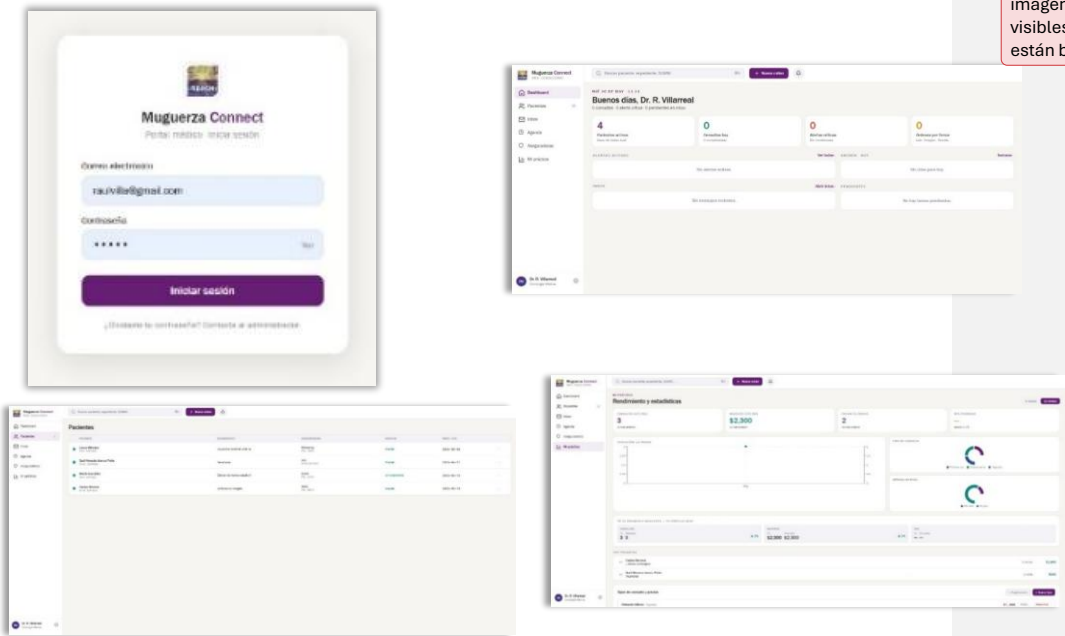
Muguerza Connect is not being proposed as a day-one fix for the entire existing system. Rather, the current system exposes the problem Muguerza must avoid replicating as we build the new ambulatory strategy. The platform will enable our ambulatory clinics to streamline scheduling, insurance pre-authorization tracking, document intake, treatment coordination, results delivery, appointment reminders, follow-up, direct referral communication within Muguerza infrastructure, and additional features explored in the section “**Muguerza Connect: Technical Analysis.**”

The physician retains clinical authority while gaining operational support. The administrative team gains a structured workflow. The insurer interaction becomes trackable instead of fragmented. In this sense, **Muguerza Connect** is both a CRM and an operational platform. It manages the relationship with the patient, but it also manages the operational journey around that patient. It connects demand generation, physician referral, insurance coordination, clinical delivery, and follow-up into one digital infrastructure.

In the current scope, the platform is intended to be used initially only within the ambulatory clinics, separating the CEI ambulatory model from the private-office model. Over time, **Muguerza Connect** can become a scalable digital service for Muguerza's broader physician network, including private tenants who currently lack hospital-provided administrative technology.

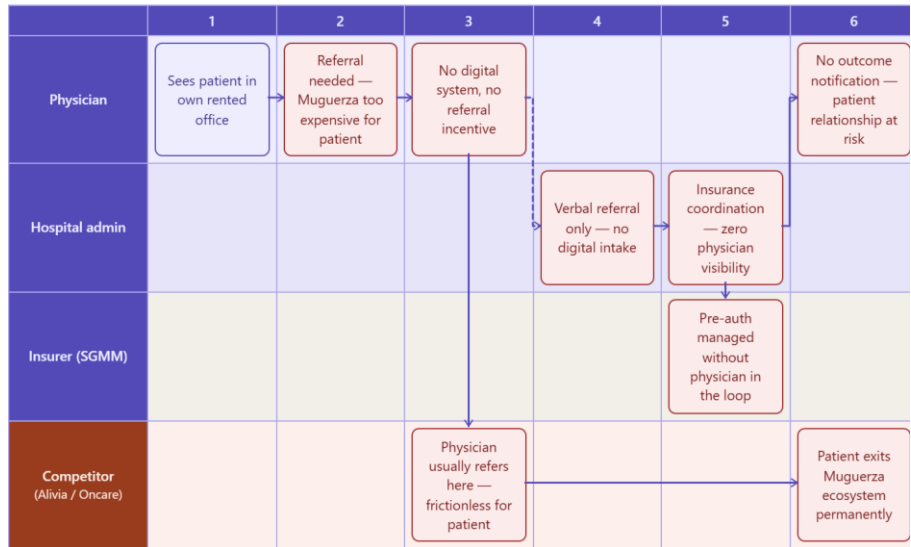
Exhibit: Muguerza connect UI

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Current physician Journey

Current state — Muguerza physician journey (today)



5.4 Specialized WhatsApp Model for Ambulatory Services serving as initial coordination layer for Muguerza Connect

One of the clearest patterns across the three benchmark models is that the most operationally efficient ambulatory platforms do not compete with clinical quality alone, they compete with excellent customer attention. And in the Mexican healthcare market the customer attention relies on a single communication channel WhatsApp.

Alivia processes its entire patient intake: scheduling, location assignment, insurance quotation, and treatment coordination, through an automated WhatsApp flow. Salud Digna routes 91% of all patient interactions, from appointment booking to results delivery, exclusively through the platform. Oncare relies on WhatsApp as the primary communication layer between patients, administrative staff, and referring physicians. The common denominator is not the technology itself, but what it solves: the volume problem.

In any ambulatory operation serving hundreds of recurring patients, manual front-desk communication becomes the first point of failure. Staff cannot respond to every inquiry in real time. Patients disengage. Appointments are missing. Referrals leak. The competitors that solved this problem did not hire more administrative staff – they automated the front door.

Muguerza Concierge is the CEI proposal for Muguerza's WhatsApp-native communication layer. It is not a standalone chatbot. It is directly connected to Muguerza Connect and serves as the first intake layer for our ambulatory patients. This Chatbot can Schedule, answer frequent questions, indicate current locations and prices, see doctor availabilities, without the need of initial front desk interaction.

The model serves:

- **Institutional ambulatory patients** accessing the full ambulatory service offering: infusions, laboratory, imaging, specialty consultations, and surgical coordination.

The architecture is intentionally simple. No app download. No registration wall. The patient opens WhatsApp, initiates contact, and our IA concierge handles the rest, questions, availability, confirmation, reminders, and results notification. with human escalation available at any point for complex cases.

Muguerza Concierge does not replace the administrative team. It protects their capacity for the interactions that require judgment – insurer coordination, clinical escalation, exception handling – by absorbing the high-volume, low-complexity interactions that currently consume their time and degrade the patient experience.

Together, Muguerza Connect and Muguerza Concierge form a two-layer digital infrastructure: Connect manages the operational backend – workflows, pre-authorization, clinical coordination, and insurer tracking – while Concierge manages the relationship layer that patients and physicians experience.

In its initial deployment, Muguerza Concierge operates exclusively within the ambulatory platform. Over time, the same infrastructure can be extended to Muguerza's broader private physician network converting a competitive differentiator of the ambulatory model into a scalable digital service for the full Muguerza ecosystem. ([Link to demo Video](#))

4.5 Patient and physician journeys: Experience flowchart for our ambulatory model

The ambulatory experience is separated into 3 distinct modules our WhatsApp communication channel, which is directly connected to Muguerza Connect, our own physician faced CRM, and our operation at the new fully integrated ambulatory Clinics.

The following link includes a detailed patient and physician experience flows, operational touchpoints, escalation paths, document intake appointment scheduling, reminders, results delivery, check-in, CRM workflows, and clinic variants for both existing hospitals and new ambulatory spokes. The goal is to show how the digital and physical components work together as one integrated ambulatory model, from first patient contact to clinical follow-up and operational execution.

[Digital Layer Ambulatory Model – Journey and flowcharts for WhatsApp Concierge and Muguerza Connect](#)

- To take an in-depth look of our WhatsApp journey for patients Click the “WhatsApp – Paciente” section
- To see our Muguerza Connect flows and understand the function of the platform Click “Connect-CRM” section
- To check out the mapped process for our clinics Click “Clínica operación” Section

6. Inorganic growth: Saltillo Clinic, roadmap and business logic

With all the laid-out pieces we are begging to map the implementation of these new ambulatory clinics, modeling how these clinics will operate and the physical and technical requirements for the correct introduction of the ambulatory model.

5.1 Saltillo pipeline

[Info de Armando](#)

5.2 Implementation roadmap

Ya con la info puedo armar un roadmap y de ahí corroborar con el equipo para ver que tan convincente es etc.

5.3 Ambulatory Design sketch

Imagen de cómo se vería una clínica, y agrego en un link el journey que ya se tiene del proceso en clínica

[Journey at Clinic](#)

Comentado [SJ12]: Hay algunas cosas de flujo de Connect que se tienen que arreglar o dejar mas claro que no me convencen o no muestran la funcionalidad completa

Comentado [SJ13]: Cambiar a ingles olvide este punto

Comentado [SJ14]: @Jose Armando Dorbecker Herrera en la sección de 5 se ocupa la información recabada de saltillo para explorar mas sobre la posibilidad tecnica, ventajas y roadmap de implementacion del proyecto

5. Technical analysis

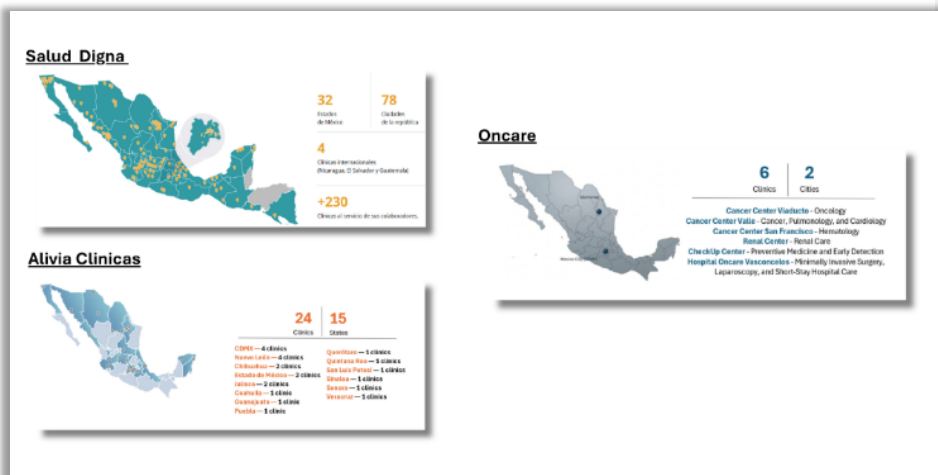
The ambulatory Proposal already established includes several digital innovations that will make this business model disruptive and difficult to replicate making Muguerza the leaders in ambulatory services at Northern Mexico, capturing and retaining not only patients but physicians making our digital layer indispensable for our stakeholders when searching for ambulatory Services.

Muguerza Connect: Business Requirements

Digital

6. Appendix

The big three geographical footprint



DEMO EXAMPLE Private Office bot – Developed by Business development team member